

Boulder Emotional Wellness

Date and Time: 2/10/2025 11:44 AM

Clinician: Niccole Fortunato

Supervisor: Erica Johnson

Patient: Amanda Duffy, DOB 1/14/1998

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Presenting Problem

Client reports "generalized anxiety related to romantic relationships, health anxiety related to diagnosis, death anxiety connected to health diagnosis and the state of the world." Client reports "noticing an impact on the people closest to me." Client reports "anxiety started end of HS, early college 2016/2017 time frame." Client reports "I am more anxious when I have more free time that's when anxious thoughts creep in." Client reports "my anxiety manifests more in the morning when I wake up."

Treatment Goal

Client would like to decrease anxiety and increase interpersonal relationship communication skills.

Objective 1

Client will learn and implement self-soothing techniques at least 3x a week in order to decrease "anxiety" while increasing emotional regulation aeb client report in session.

Estimated Completion: 6 months (8/10/2025)

Treatment Strategy / Intervention

Boulder Emotional Wellness

Date and Time: 2/10/2025 11:44 AM

Clinician: Niccole Fortunato

Supervisor: Erica Johnson

Patient: Amanda Duffy, DOB 1/14/1998

-
- Cognitive Challenging.
 - Cognitive Refocusing.
 - Cognitive Reframing.
 - Communication Skills.
 - EMDR.
 - EMDR grounding/resourcing exercises.
 - EMDR Prep.
 - Experiential Somatic Exercise.
 - Exploration of Coping Patterns.
 - Exploration of Emotions.
 - Exploration of Nervous System Activation.
 - Exploration of Relationship Patterns.
 - Grounding/ resourcing.
 - Guided Imagery.
 - Interactive Feedback.
 - Interpersonal Resolutions.
 - Intrapersonal Resolutions.
 - Mindfulness Training.
 - Nervous System Exercise.
 - Nervous System Mapping.
 - Open Ended Questioning.
 - Parts work.
 - Preventative Services.
 - Psycho-Education.
 - Relaxation/Deep Breathing.
 - Role-Play/Behavioral Rehearsal.
 - Structured Problem Solving.
 - Supportive Reflection.
 - Symptom Management.

Objective 2

Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her anxiety in the moment and client report in session.

Estimated Completion: 6 months (8/10/2025)

Treatment Strategy / Intervention

Boulder Emotional Wellness

Date and Time: 2/10/2025 11:44 AM

Clinician: Niccole Fortunato

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Patient: Amanda Duffy, DOB 1/14/1998

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 - Open Ended Questioning.
 - Parts work.
 - Preventative Services.
 - Psycho-Education.
 - Relaxation/Deep Breathing.
 - Role-Play/Behavioral Rehearsal.
 - Structured Problem Solving.
 - Supportive Reflection.
 - Symptom Management.

Objective 3

Client will practice communication skills in session to decrease anxiety in order to engage in social interactions in interpersonal relationships from a space of confidence and strength as client report in session.

Estimated Completion: 6 months (8/10/2025)

Treatment Strategy / Intervention

- Cognitive Challenging.

Boulder Emotional Wellness

Date and Time: 2/10/2025 11:44 AM

Clinician: Niccole Fortunato

Supervisor: Erica Johnson

Patient: Amanda Duffy, DOB 1/14/1998

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- Cognitive Refocusing.
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 - Open Ended Questioning.
 - Preventative Services.
 - Parts work.
 - Psycho-Education.
 - Relaxation/Deep Breathing.
 - Role-Play/Behavioral Rehearsal.
 - Structured Problem Solving.
 - Supportive Reflection.
 - Symptom Management.

Objective 4

There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Estimated Completion: 6 months (8/10/2025)

Prescribed Frequency of Treatment

Treatment Plan

Boulder Emotional Wellness

Date and Time: 2/10/2025 11:44 AM

Clinician: Nicole Fortunato

Supervisor: Erica Johnson

Patient: Amanda Duffy, DOB 1/14/1998

Weekly

I declare that these services are medically necessary and appropriate to the recipient's diagnosis and needs.

Nicole Fortunato, Associate, signed this note and declared this information to be accurate and complete on 2/10/2025 at 1:03 PM.

Erica Johnson, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 2/11/2025 at 11:58 AM.

Signature

I acknowledge that I have participated in the development of the treatment plan and agree with the goals, objectives, and interventions.

A handwritten signature in black ink, appearing to read 'Amanda Nicole Duffy', with a stylized, cursive script.

Amanda Nicole Duffy
2/11/2025

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 5/1/2025 9:03 AM - 9:31 AM

Duration: 28 minutes

Service Code: 90832

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Euthymic

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 5/1/2025 9:03 AM - 9:31 AM

Duration: 28 minutes

Service Code: 90832

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "Wellbutrin 300 mgs for anxiety PCP Christa Toomre is the prescribing physician." Client reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." Client reports "I also take a ton of Vitamins." Client reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

Client reports "I don't feel safe around him" in regards to relationship dynamic where her partner goes back and forth between wanting connection and disconnecting. Client reports "if it ends I feel like it failed." Client reports "I'm scared to let go because I don't believe things will get better."

Objective Content

Therapist held space for client to check in. Client shared that she wasn't feeling well but asked if she needed to leave early could she. Therapist gave client permission to leave early or not have the session if that's what was needed. Client stated "I'll stay for as long as I can." Therapist honored client's need. Client then shared about relationship dynamics she is continuing to navigate. Therapist listened as client shared. Client spoke to the mental and emotional impact this dynamic has on her. Therapist offered emotional validation and supportive reflections. Client then spoke to feeling "distracted and struggling to focus." Therapist offered psych-ed and asked open ended questions. Client then shared "my head is pounding. I need to leave now." Therapist honored client's need and ended session early.

Interventions Used

The following interventions were used: Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Cognitive Challenging, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques at least 3x a week in order to decrease "anxiety" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her anxiety in the moment aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 5/1/2025 9:03 AM - 9:31 AM

Duration: 28 minutes

Service Code: 90832

Location: Teletherapy- Patient is Home

Participants: Client Only

-
3. Client will practice communication skills in session to decrease anxiety in order to engage in social interactions in interpersonal relationships from a space of confidence and strength aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was at home for telehealth session.

Plan

Therapist and client will continue exploring client's patterning, grounding and resourcing her anxiety, and figure out ways to meet her needs interpersonally and intrapersonally. As well as explore client's parts using parts work.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Niccole Fortunato, Associate, signed this note and declared this information to be accurate and complete on 5/1/2025 at 10:25 AM.

Erica Johnson, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 5/2/2025 at 2:26 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 6/5/2025 9:05 AM - 9:58 AM

Duration: 53 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Euthymic

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 6/5/2025 9:05 AM - 9:58 AM

Duration: 53 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "Wellbutrin 300 mgs for anxiety PCP Christa Toomre is the prescribing physician." Client reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." Client reports "I also take a ton of Vitamins." Client reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

Client reports feeling "annoyed." Client reports "it feels embarrassing and frustrating." Client reports "it's proof that something is wrong with me."

Objective Content

Therapist held space for client to check in. Client shared how she was feeling and updates on her health journey with MS. Therapist listened as client shared. Client shared new news and information she has received about her diagnosis, relational support she has received, and the impacts of side effects from medication. Therapist offered supportive reflections, emotional validation, and asked open ended questions. Client talked more about the relationships that have been supportive and fears around meeting and sharing this part of herself with someone new. Therapist continued to ask open ended questions and offer emotional validation. Along with offering reframes, psych-ed, and checking with client around how it was to share all that she did and how she was received. Client responded to therapist inquires and expressed gratitude for therapist holding space for this conversation.

Interventions Used

The following interventions were used: Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Cognitive Challenging, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques at least 3x a week in order to decrease "anxiety" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her anxiety in the moment aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 6/5/2025 9:05 AM - 9:58 AM

Duration: 53 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

-
3. Client will practice communication skills in session to decrease anxiety in order to engage in social interactions in interpersonal relationships from a space of confidence and strength aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was at home for telehealth session.

Plan

Therapist and client will continue exploring client's patterning, grounding and resourcing her anxiety, and figure out ways to meet her needs interpersonally and intrapersonally. As well as explore client's parts using parts work. As well as, continue to explore and speak to client's experience with MS.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Niccole Fortunato, Associate, signed this note and declared this information to be accurate and complete on 6/6/2025 at 8:42 PM.

Erica Johnson, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 6/10/2025 at 8:24 AM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 6/12/2025 9:01 AM - 10:01 AM

Duration: 60 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Euthymic

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 6/12/2025 9:01 AM - 10:01 AM

Duration: 60 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "Wellbutrin 300 mgs for anxiety PCP Christa Toomre is the prescribing physician." Client reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." Client reports "I also take a ton of Vitamins." Client reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

Client reports "I spent all week panicked and stressed." Client reports "perception means lack of control and lack of empathy from the other person." Client reports "I want flexibility and trust that is the part that pisses me off."

Objective Content

Therapist held space for client to check in. Client shared an experience she had at work. Therapist listened as client shared. Client shared the mental, physical, and emotional impacts of the experience she recently had. Therapist offered supportive reflections, emotional validation, and asked open ended questions. Client talked more about the intra personal conflicts and desires that surfaced and how this experience offered clarity into some questions she has been having. Therapist offered psych-ed, reframes, and continued to ask open ended questions and offer emotional validation. Client responded to therapist inquires. Therapist and client ended with talking about an exciting trip client has coming up and things she is looking forward to.

Interventions Used

The following interventions were used: Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Cognitive Challenging, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques at least 3x a week in order to decrease "anxiety" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her anxiety in the moment aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 6/12/2025 9:01 AM - 10:01 AM

Duration: 60 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

-
3. Client will practice communication skills in session to decrease anxiety in order to engage in social interactions in interpersonal relationships from a space of confidence and strength aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was at home for telehealth session.

Plan

Therapist and client will continue exploring client's patterning, grounding and resourcing her anxiety, and figure out ways to meet her needs inter personally and intra personally. As well as explore client's parts using parts work. As well as, continue to explore and speak to client's experience with MS.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, Associate, signed this note and declared this information to be accurate and complete on 6/14/2025 at 7:02 AM.

Erica Johnson, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 6/16/2025 at 4:25 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 6/23/2025 10:32 AM - 11:29 AM

Duration: 57 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 6/23/2025 10:32 AM - 11:29 AM

Duration: 57 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "Wellbutrin 300 mgs for anxiety PCP Christa Toomre is the prescribing physician." Client reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." Client reports "I also take a ton of Vitamins." Client reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

Client reports "I was anxious and had a panic attack." Client reports "I was wanting reassurance or offering a disclaimer." Client reports "I am nervous about my MRI results."

Objective Content

Therapist held space for client to check in. Client shared how her trip went and what she noticed about an internal dynamic that arose within an interpersonal relationship dynamic. Therapist listened as client shared and asked open ended questions. Client responded to therapist questions and then moved the subject into how she is feeling about an area connected to her MS diagnosis. Therapist offered supportive reflections and emotional validation. Client continued to speak to the mental, physical, and emotional impacts of the diagnosis. Therapist continued to offer emotional validation. As well as offering psych-ed, reframes, and observations. Therapist and client uncovered a need for client around being more able to offer more self-regulation than needing co-regulation. Therapist and client will work on and find ways for client to build more tools and resourcing to cultivate "internal safety."

Interventions Used

The following interventions were used: Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Cognitive Challenging, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques at least 3x a week in order to decrease "anxiety" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her anxiety in the moment aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 6/23/2025 10:32 AM - 11:29 AM

Duration: 57 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

-
3. Client will practice communication skills in session to decrease anxiety in order to engage in social interactions in interpersonal relationships from a space of confidence and strength aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was at home for telehealth session.

Plan

Therapist and client will continue exploring client's patterning, grounding and resourcing her anxiety, and figure out ways to meet her needs inter personally and intra personally. As well as explore client's parts using parts work. Therapist will hold space for client to continue to explore and speak to client's experience with MS. Along with building client's capacity for internal resourcing to be less reliant on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, Associate, signed this note and declared this information to be accurate and complete on 6/23/2025 at 3:40 PM.

Erica Johnson, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 6/24/2025 at 4:43 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 7/3/2025 9:18 AM - 10:01 AM

Duration: 43 minutes

Service Code: 90834

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 7/3/2025 9:18 AM - 10:01 AM

Duration: 43 minutes

Service Code: 90834

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "Wellbutrin 300 mgs for anxiety PCP Christa Toomre is the prescribing physician." Client reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." Client reports "I also take a ton of Vitamins." Client reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

Client reports feeling "frustrated and discombobulated." Client reports "at it's core is being really hard on myself" in reference to the discontentment and comparison. Client reports "I've been in freeze a lot lately."

Objective Content

Therapist held space for client to check in. Client shared how she was feeling and updates on her health journey with MS. Therapist listened as client shared. Client then spoke to patterns she notices around feeling "discontent and comparing." Therapist and client explored client's discontent and comparison behaviors/ mechanisms. Client shared where she notices the thought patterns and actions or lack there of come from. Therapist asked open ended questions, offered supportive reflections, emotional validation, and psych-ed. Therapist and client then explored client's attachment styles and which ones she notices is at play in multiple areas of her life. Client expressed resonance for psych-ed on attachment styles. Therapist gave client HW to track her patterns using the psych-ed offered in session. Client expressed willingness to engage with HW outside of session and will report back.

Interventions Used

The following interventions were used: Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Cognitive Challenging, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques at least 3x a week in order to decrease "anxiety" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her anxiety in the moment aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 7/3/2025 9:18 AM - 10:01 AM

Duration: 43 minutes

Service Code: 90834

Location: Teletherapy- Patient is Home

Participants: Client Only

-
3. Client will practice communication skills in session to decrease anxiety in order to engage in social interactions in interpersonal relationships from a space of confidence and strength aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was at home for telehealth session.

Plan

Therapist and client will continue exploring client's patterning, grounding and resourcing her anxiety, and figure out ways to meet her needs inter personally and intra personally. As well as explore client's parts using parts work. Therapist will hold space for client to continue to explore and speak to client's experience with MS. Along with building client's capacity for internal resourcing to be less reliant on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Niccole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 7/17/2025 at 3:15 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 7/18/2025 at 2:55 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 7/10/2025 9:02 AM - 10:00 AM

Duration: 58 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Euthymic

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 7/10/2025 9:02 AM - 10:00 AM

Duration: 58 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "Wellbutrin 300 mgs for anxiety PCP Christa Toomre is the prescribing physician." Client reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." Client reports "I also take a ton of Vitamins." Client reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

Client reports "I would like to prioritize better habits." Client reports being able to verbalize a need of hers in the moment in an interpersonal relationship. Client expressed being proud of herself for advocating for her needs. Client reports "it makes me feel sad, anxious, and guilty" to think about how some of the interactions with her parents have gone. Client spoke to what happens in her brain and body when she is with her parents for an extended period of time.

Objective Content

Therapist held space for client to check in. Client shared how she was feeling and what she noticed about a habit of hers. Therapist listened as client shared and notices client struggles to go towards her emotional experience when she is not activated. Therapist brought this pattern to client's attention. Client spoke to what she notices about why that is. Client speaks to intrapersonal dynamics that leads to that response. Therapist asked open ended questions, offered supportive reflections, and therapist observations. Client spoke to relationship dynamics and patterns she is noticing those dynamics with her family. Therapist offered psych-ed, cognitive challenging and reframing, emotional validation, and asked open ended questions. Client responded and offered when things landed and when they didn't. Therapist and client spoke about dynamics within the therapeutic relationship as well. Therapist brought dynamics to client's attention after asking for consent. Client was open to therapist observations and stated what it brought up for her.

Interventions Used

The following interventions were used: Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Cognitive Challenging, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques at least 3x a week in order to decrease "anxiety" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 7/10/2025 9:02 AM - 10:00 AM

Duration: 58 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

implement new coping skills and regulate her anxiety in the moment aeb client report in session.

Progress: Progressing

3. Client will practice communication skills in session to decrease anxiety in order to engage in social interactions in interpersonal relationships from a space of confidence and strength aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was at home for telehealth session.

Plan

Therapist and client will continue exploring client's patterning, grounding and resourcing her anxiety, and figure out ways to meet her needs inter personally and intra personally. As well as explore client's parts using parts work. Therapist will hold space for client to continue to explore and speak to client's experience with MS. Along with building client's capacity for internal resourcing to be less reliant on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 7/11/2025 at 2:59 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 7/11/2025 at 4:46 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 7/17/2025 9:05 AM - 10:00 AM

Duration: 55 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 7/17/2025 9:05 AM - 10:00 AM

Duration: 55 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "Wellbutrin 300 mgs for anxiety PCP Christa Toomre is the prescribing physician." Client reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." Client reports "I also take a ton of Vitamins." Client reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

Client reports "I have been recognizing the ways I am showing up in my relationship and the way's I have not been. I have been feeling guilt and regret for the ways I haven't been showing up." Client reports "it's confusing and I have conflicting parts." Client reports "I notice I am always wishing for the next thing" client speaking to her systems automatic pattern to go into a wishing defense and not be able to stay or engage in the present moment. Client asked "what's the difference between reflecting and dwelling."

Objective Content

Therapist held space for client to check in. Client shared how she was feeling and patterns she is becoming more aware of through our work. Therapist listened as client shared. Client spoke to the intrapersonal and interpersonal dynamics she has been noticing and how the two are interconnected. Therapist asked open ended questions and offered supportive reflections. Therapist and client explored client's patterning and her coping strategies within the patterning. Client is able to recognize how her patterning isn't always serving her and the stories/ beliefs that are held in the patterning connected to her emotions and emotional reflection. Client expressed her fears and worries about getting "stuck or dwelling." Therapist offered emotional validation, psych-ed, reframes, and ways to work with grounding and resourcing to combat her fears. Client was open and receptive to therapist offerings and expressed resonance and understanding and named the ways she will implement the information offered and report back what she notices about the shifts she makes.

Interventions Used

The following interventions were used: Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Cognitive Challenging, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, Grounding/ resourcing, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques at least 3x a week in order to decrease "anxiety" while increasing emotional regulation aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 7/17/2025 9:05 AM - 10:00 AM

Duration: 55 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

-
2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her anxiety in the moment aeb client report in session.

Progress: Progressing

3. Client will practice communication skills in session to decrease anxiety in order to engage in social interactions in interpersonal relationships from a space of confidence and strength aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was at home for telehealth session.

Plan

Therapist and client will continue exploring client's patterning, grounding and resourcing her anxiety, and figure out ways to meet her needs inter personally and intra personally. As well as explore client's parts using parts work. Therapist will hold space for client to continue to explore and speak to client's experience with MS. Along with building client's capacity for internal resourcing to be less reliant on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 7/17/2025 at 1:10 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 7/18/2025 at 2:23 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 7/24/2025 9:00 AM - 9:57 AM

Duration: 57 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 7/24/2025 9:00 AM - 9:57 AM

Duration: 57 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "Wellbutrin 300 mgs for anxiety PCP Christa Toomre is the prescribing physician." Client reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." Client reports "I also take a ton of Vitamins." Client reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

Client reports "I got sad and angry and I am afraid of being left behind and alone forever" client's stories, beliefs, and fears. Client reports "I realize I need validation from men and it feels so personal." Client reports "I feel lost and not confident in myself or my choices" the impact of the ending.

Objective Content

Therapist held space for client to check in. Client shared the impact of realizing her relationship with her partner is no longer working or moving forward. Therapist listened as client shared about what occurred and how her and her partner came to the decision to end things. As client shared she expressed her sadness and grief through crying. Therapist offered emotional validation, supportive reflections, and asked open ended questions. Client shared the stories, beliefs, and fears that are all at the forefront as a result of the ending. Therapist offered reframes, psych-ed, and used parts work. Client engaged with interventions used and expressed feeling "better" by the end of session.

Interventions Used

The following interventions were used: Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Cognitive Challenging, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, Grounding/ resourcing, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques at least 3x a week in order to decrease "anxiety" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her anxiety in the moment aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 7/24/2025 9:00 AM - 9:57 AM

Duration: 57 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

-
3. Client will practice communication skills in session to decrease anxiety in order to engage in social interactions in interpersonal relationships from a space of confidence and strength aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was at home for telehealth session.

Plan

Therapist and client will continue exploring client's patterning, grounding and resourcing her anxiety, and figure out ways to meet her needs inter personally and intra personally. As well as explore client's parts using parts work. Therapist will hold space for client to continue to explore and speak to client's experience with MS. Along with building client's capacity for internal resourcing to be less reliant on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Niccole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 7/28/2025 at 1:29 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 7/29/2025 at 5:15 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 7/30/2025 4:05 PM - 5:00 PM

Duration: 55 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 7/30/2025 4:05 PM - 5:00 PM

Duration: 55 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "Wellbutrin 300 mgs for anxiety PCP Christa Toomre is the prescribing physician." Client reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." Client reports "I also take a ton of Vitamins." Client reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

Client reports "I am getting my infusion tomorrow and I don't know who I want to support me afterwards" client is trying to decide if she will use her ex as a support or if she will implement other relationships for that role. Client reports "the big thing I noticed in both was how important it is for me to have someone with me afterwards" one of the take away from the exercise client participated in. Client reports "the drive home is where the longing for my ex comes in" another insight client took away from the exercise.

Objective Content

Therapist held space for client to check in. Client shared how she was feeling and spoke to everything that is coming up for her around her next infusion for MS. Therapist listened as client shared. Client expressed her sadness through tears throughout session as shared all of the different relational dynamics and components of what her infusion day holds for her. Therapist asked open ended questions, offered supportive reflections, emotional validation, and psych-ed. Therapist then invited client to practice an exercise to work with two different scenarios surrounding choices she has to make regarding the support she will receive after her infusion. Client was open to participating in the exercise. Therapist offered two different scenarios based on what client was sharing and had client track how her system responded to both scenarios. Client shared what she noticed come up in both. Therapist and client found the important through lines. Therapist offered tools and resources and ways to work with getting her needs met. Client was open and receptive to therapist offerings.

Interventions Used

The following interventions were used: Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Cognitive Challenging, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, Grounding/ resourcing, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques at least 3x a week in order to decrease "anxiety" while increasing emotional regulation aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 7/30/2025 4:05 PM - 5:00 PM

Duration: 55 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

-
2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her anxiety in the moment aeb client report in session.

Progress: Progressing

3. Client will practice communication skills in session to decrease anxiety in order to engage in social interactions in interpersonal relationships from a space of confidence and strength aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was at home for telehealth session.

Plan

Therapist and client will continue exploring client's patterning, grounding and resourcing her anxiety, and figure out ways to meet her needs inter personally and intra personally. As well as explore client's parts using parts work. Therapist will hold space for client to continue to explore and speak to client's experience with MS. Along with building client's capacity for internal resourcing to be less reliant on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 7/30/2025 at 5:10 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 7/31/2025 at 7:41 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 8/7/2025 9:02 AM - 10:00 AM

Duration: 58 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 8/7/2025 9:02 AM - 10:00 AM

Duration: 58 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "Wellbutrin 300 mgs for anxiety PCP Christa Toomre is the prescribing physician." Client reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." Client reports "I also take a ton of Vitamins." Client reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

Client reports "I was upset, emotional, and felt abandoned" how she relates to a relationship dynamic. Client reports "I was shocked and confused" how she felt about a relationship dynamic. Client reports "I feel it in my chest and my shoulders and there is a pit in my stomach and I feel heavy."

Objective Content

Therapist held space for client to check in. Client shared how she was feelings and spoke to a current relationship dynamic that is causing her distress. Therapist listened as client shared. Therapist and client explored client's relationship dynamic. Client spoke to the younger parts, stories, and beliefs she is having in connection to the dynamic. Therapist offered supportive reflections, emotional validation, and asked open ended questions. Therapist and client explored other areas of her life that have felt "stressful" this week. Client shared the stressors and the way her system respond to the stressors. Therapist offered psych-ed, reframes, and continued to offer emotional validation. Therapist invited client to practice somatic based exercises to work with the physical sensations in her body connected to the stress. Client participated in exercises and shared what she noticed come up for her throughout the exercises.

Interventions Used

The following interventions were used: Cognitive Reframing, Structured Problem Solving, Relaxation/Deep Breathing, Cognitive Refocusing, Exploration of Emotions, Cognitive Challenging, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Experiential Somatic Exercise, Open Ended Questioning, Exploration of Nervous System Activation, Grounding/ resourcing, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques at least 3x a week in order to decrease "anxiety" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her anxiety in the moment aeb client report in session.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 8/7/2025 9:02 AM - 10:00 AM

Duration: 58 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Progress: Progressing

3. Client will practice communication skills in session to decrease anxiety in order to engage in social interactions in interpersonal relationships from a space of confidence and strength aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was at home for telehealth session.

Plan

Therapist and client will continue exploring client's patterning, grounding and resourcing her anxiety, and figure out ways to meet her needs inter personally and intra personally. As well as explore client's parts using parts work. Therapist will hold space for client to continue to explore and speak to client's experience with MS. Along with building client's capacity for internal resourcing to be less reliant on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 8/11/2025 at 8:48 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 8/12/2025 at 4:48 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 8/21/2025 9:02 AM - 9:58 AM

Duration: 56 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Irritable

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 8/21/2025 9:02 AM - 9:58 AM

Duration: 56 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "Wellbutrin 300 mgs for anxiety PCP Christa Toomre is the prescribing physician." Client reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." Client reports "I also take a ton of Vitamins." Client reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

Client reports "I am confused, hurt, and fixated on the why" the impact of the continued relationship dynamic causing her distress. Client reports "I feel abandoned in a lot of ways" the impact of the other person in the relationship cutting contact with no explanation. Client reports "it's exhausting, I am frustrated, and over it" the emotional impact and how client responds to the frustration.

Objective Content

Therapist held space for client to check in. Client shared how she was feeling and continued to speak to the relationship dynamic that occurred a few weeks ago that is still causing her distress. Therapist listened as client shared. Therapist and client explored client's relationship dynamic. Client spoke to the younger parts, stories, and beliefs she is having in connection to the dynamic. Therapist offered supportive reflections, emotional validation, asked open ended questions, and used parts work. Therapist and client explored ways to resource her younger parts and the "abandonment" she is feeling as the relationship dynamic persists. Client shared her "sadness and frustration" and named a pattern of hers connected to finding "security" in her relationships rather than in herself. Therapist explored client's pattern of finding security outside of herself. Therapist offered psych-ed, reframes, and grounding and resourcing tools to implement when working with her younger parts and patterns. Client was open and receptive to therapist offerings.

Interventions Used

The following interventions were used: Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Cognitive Challenging, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, Grounding/ resourcing, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques at least 3x a week in order to decrease "anxiety" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 8/21/2025 9:02 AM - 9:58 AM

Duration: 56 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

implement new coping skills and regulate her anxiety in the moment aeb client report in session.

Progress: Progressing

3. Client will practice communication skills in session to decrease anxiety in order to engage in social interactions in interpersonal relationships from a space of confidence and strength aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was at home for telehealth session.

Plan

Therapist and client will continue exploring client's patterning, grounding and resourcing her anxiety, and figure out ways to meet her needs inter personally and intra personally. As well as explore client's parts using parts work. Therapist will hold space for client to continue to explore and speak to client's experience with MS. Along with building client's capacity for internal resourcing to be less reliant on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 8/22/2025 at 9:30 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 8/25/2025 at 4:23 PM.

Boulder Emotional Wellness

Date and Time: 8/25/2025 6:45 PM

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Presenting Problem

Client reports "generalized anxiety related to romantic relationships, health anxiety related to diagnosis, death anxiety connected to health diagnosis and the state of the world." Client reports "noticing an impact on the people closest to me." Client reports "anxiety started end of HS, early college 2016/2017 time frame." Client reports "I am more anxious when I have more free time that's when anxious thoughts creep in." Client reports "my anxiety manifests more in the morning when I wake up." Client reports "the obsessive thoughts and rumination on all of these areas in my life. I get stuck in those thought loops and don't know how to get out."

Treatment Goal

Client would like to decrease "anxiety and overthinking" and increase interpersonal relationship communication skills. Client would like to decrease "panic, anxiety, and stress" connected to her MS chronic illness diagnosis as well as to increase confidence and skills to navigate the unpredictability that comes along with having MS.

Objective 1

Boulder Emotional Wellness

Date and Time: 8/25/2025 6:45 PM

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease "anxiety, panic, and stress" connected to MS diagnosis. Client will decrease her distress level from a 8 to a level 7 over the next six months aeb client report in session.

Estimated Completion: 6 months (2/25/2026)

Treatment Strategy / Intervention

- Cognitive Challenging.
- Cognitive Refocusing.
- Cognitive Reframing.
- Communication Skills.
- EMDR.
- EMDR grounding/resourcing exercises.
- EMDR Prep.
- Experiential Somatic Exercise.
- Exploration of Coping Patterns.
- Exploration of Emotions.
- Exploration of Nervous System Activation.
- Exploration of Relationship Patterns.
- Grounding/ resourcing.
- Guided Imagery.
- Interactive Feedback.
- Intrapersonal Resolutions.
- Mindfulness Training.
- Nervous System Exercise.
- Nervous System Mapping.
- Open Ended Questioning.
- Parts work.
- Preventative Services.
- Psycho-Education.
- Relaxation/Deep Breathing.
- Role-Play/Behavioral Rehearsal.
- Structured Problem Solving.
- Supportive Reflection.
- Symptom Management.

Objective 2

Boulder Emotional Wellness

Date and Time: 8/25/2025 6:45 PM

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her NS in the moment. Client will be tracking her triggers to be able to resource and ground herself when triggered from a distress level of 8 to a level 7 over the next six months aeb client report in session.

Estimated Completion: 6 months (2/25/2026)

Treatment Strategy / Intervention

- Cognitive Challenging.
- Cognitive Refocusing.
- Cognitive Reframing.
- Communication Skills.
- EMDR.
- EMDR grounding/resourcing exercises.
- EMDR Prep.
- Experiential Somatic Exercise.
- Exploration of Coping Patterns.
- Exploration of Emotions.
- Exploration of Nervous System Activation.
- Exploration of Relationship Patterns.
- Grounding/ resourcing.
- Guided Imagery.
- Interactive Feedback.
- Interpersonal Resolutions.
- Intrapersonal Resolutions.
- Mindfulness Training.
- Nervous System Exercise.
- Nervous System Mapping.
- Open Ended Questioning.
- Parts work.
- Preventative Services.
- Psycho-Education.
- Relaxation/Deep Breathing.
- Role-Play/Behavioral Rehearsal.
- Structured Problem Solving.
- Supportive Reflection.
- Symptom Management.

Boulder Emotional Wellness

Date and Time: 8/25/2025 6:45 PM

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Objective 3

Client will explore and use cognitive reframing around "obsessive/ ruminating thought" patterns that are currently at distress level of 10. Client will increase her awareness and ability to challenge her negative thought patterns and offer reframes to decrease distressing thought patterns from a level 10 to 9 over the next six months aeb client report in session.

Estimated Completion: 6 months (2/25/2026)

Treatment Strategy / Intervention

- Cognitive Challenging.
- Cognitive Refocusing.
- Cognitive Reframing.
- Communication Skills.
- EMDR.
- Experiential Somatic Exercise.
- Exploration of Coping Patterns.
- Exploration of Emotions.
- Exploration of Nervous System Activation.
- Exploration of Relationship Patterns.
- Grounding/ resourcing.
- Interactive Feedback.
- Interpersonal Resolutions.
- Intrapersonal Resolutions.
- Mindfulness Training.
- Open Ended Questioning.
- Preventative Services.
- Parts work.
- Psycho-Education.
- Relaxation/Deep Breathing.
- Structured Problem Solving.
- Supportive Reflection.
- Symptom Management.

Objective 4

Therapist will provide client with PRI, attachment, parts work, and nervous system regulation psychoeducation and tools to use to increase client's awareness of the trigger's in her system and decrease client's "anxiety, overthinking, and self-consciousness" connected to interpersonal relationship dynamics. Client will decrease her distress level from a level 6 to a level 5 over the next 6 months aeb client report in session.

Estimated Completion: 6 months (2/25/2026)

Boulder Emotional Wellness

Date and Time: 8/25/2025 6:45 PM

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Treatment Strategy / Intervention

- Cognitive Challenging.
- Cognitive Refocusing.
- Cognitive Reframing.
- Experiential Somatic Exercise.
- Exploration of Coping Patterns.
- Exploration of Emotions.
- Exploration of Nervous System Activation.
- Exploration of Relationship Patterns.
- Grounding/ resourcing.
- Guided Imagery.
- Interactive Feedback.
- Intrapersonal Resolutions.
- Mindfulness Training.
- Open Ended Questioning.
- Parts work.
- Preventative Services.
- Practice Nonviolent Communication.
- Psycho-Education.
- Relaxation/Deep Breathing.
- Role-Play/Behavioral Rehearsal.
- Structured Problem Solving.
- Supportive Reflection.
- Symptom Management.

Objective 5

Client reports "being told by doctors the physical manifestation of stress can have a negative impact on client's well being has led to an increase in "fear and worry" around MS diagnosis. Client will continue to learn and implement stress reduction tools and resources to decrease the "fear and worry" from a distress level of 10 to a 9 over the next six months aeb client report in session.

Estimated Completion: 6 months (2/25/2026)

Treatment Strategy / Intervention

- Cognitive Challenging.
- Cognitive Refocusing.
- Cognitive Reframing.
- Communication Skills.
- Experiential Somatic Exercise.

Treatment Plan

Boulder Emotional Wellness

Date and Time: 8/25/2025 6:45 PM

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

- Exploration of Coping Patterns.
- Exploration of Emotions.
- Exploration of Nervous System Activation.
- Exploration of Relationship Patterns.
- Grounding/ resourcing.
- Guided Imagery.
- Intrapersonal Resolutions.
- Mindfulness Training.
- Nervous System Exercise.
- Open Ended Questioning.
- Parts work.
- Preventative Services.
- Psycho-Education.
- Relaxation/Deep Breathing.
- Role-Play/Behavioral Rehearsal.
- Structured Problem Solving.
- Supportive Reflection.
- Symptom Management.

Objective 6

There are currently no obstacles to treatment regarding cultural, power, or other dynamics in therapeutic relationship and client report in session.

Additional Information

Client reports noticing a decrease in symptoms of "anxiety, stress, and panic" however, noted she still experiences high levels of "anxiety, stress, and panic" when triggered by the emotional weight of living with a chronic MS diagnosis; continued therapy is needed to help client continue to work towards her goal of lowering "anxiety, stress and panic" and increasing her knowledge and application of tools and resources.

Prescribed Frequency of Treatment

Weekly

I declare that these services are medically necessary and appropriate to the recipient's diagnosis and needs.

Niccole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 9/11/2025 at 2:09 PM.

Treatment Plan

Boulder Emotional Wellness

Date and Time: 8/25/2025 6:45 PM

Clinician: Nicole Fortunato, LPC

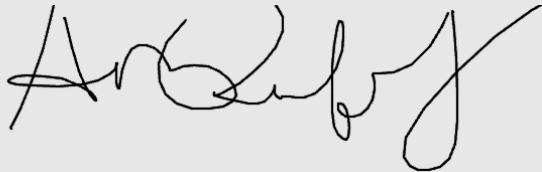
Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 9/11/2025 at 2:51 PM.

Signature

I acknowledge that I have participated in the development of the treatment plan and agree with the goals, objectives, and interventions.

A handwritten signature in black ink, appearing to read 'Amanda Duffy', with a long, sweeping horizontal stroke extending to the right.

Amanda Duffy
9/12/2025

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 8/26/2025 9:00 AM - 9:43 AM

Duration: 43 minutes

Service Code: 90834

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Euthymic

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 8/26/2025 9:00 AM - 9:43 AM

Duration: 43 minutes

Service Code: 90834

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "Wellbutrin 300 mgs for anxiety PCP Christa Toomre is the prescribing physician." Client reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." Client reports "I also take a ton of Vitamins." Client reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

Client reports "I can become combative and I can't let comments just roll off. There is a need to be heard" what client notices about a part of the pattern. Client reports "it feels like a point of connection to have someone fight for you" realization client had during exploring the pattern. Client reports "it feels like neither of us are listening to each other. I hold regret for the things I've said to my mom and dad. I would like to not keep repeating this pattern" what client notices about the pattern and the emotional impact of the pattern.

Objective Content

Client expressed needing to end session early due to preparation for upcoming trip. Therapist honored clients need to end early. Therapist then held space for client to check in. Client shared dynamics that show up with her parents and the impact those dynamics have on her. Client stated still experiencing "regret ways I've interacted with my parents in the past and I would like to find ways to relate to them differently." Therapist listened as client shared, tracking themes and patterns. Therapist and client explored clients pattern with her parents and where that pattern could be stemming from through an attachment lens. Client shared the beliefs, the needs, and the difficulty that comes from trying to get out of this pattern after it's already started. Therapist offered emotional validation, supportive reflections, and asked open-ended questions. Therapist and client explored the connection between the clients pattern, her adoption experience, and her attachment style. Client shared how they all feel connected and the connections she can see based on current and past behaviors where these dynamics showed up. Therapist offered psych-ed, reframes, and used parts work, and grounding and resourcing to work with this pattern and her attachment style. Client stated "it was helpful to explore these dynamics in the way and I would like to continue to explore these dynamics further in future sessions."

Interventions Used

The following interventions were used: Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Cognitive Challenging, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, Grounding/ resourcing, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques at least 3x a week in order to decrease "anxiety" while increasing emotional regulation aeb client report in session.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 8/26/2025 9:00 AM - 9:43 AM

Duration: 43 minutes

Service Code: 90834

Location: Teletherapy- Patient is Home

Participants: Client Only

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her anxiety in the moment aeb client report in session.

Progress: Progressing

3. Client will practice communication skills in session to decrease anxiety in order to engage in social interactions in interpersonal relationships from a space of confidence and strength aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was at home for telehealth session.

Plan

Therapist and client will continue exploring client's patterning, grounding and resourcing her anxiety, and figure out ways to meet her needs inter personally and intra personally. As well as explore client's parts using parts work. Therapist will hold space for client to continue to explore and speak to client's experience with MS. Along with building client's capacity for internal resourcing to be less reliant on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 8/30/2025 at 4:46 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 9/3/2025 at 4:04 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 9/11/2025 9:02 AM - 10:05 AM

Duration: 63 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 9/11/2025 9:02 AM - 10:05 AM

Duration: 63 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "Wellbutrin 300 mgs for anxiety PCP Christa Toomre is the prescribing physician." Client reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." Client reports "I also take a ton of Vitamins." Client reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

Client reports "I got Covid but I was fine. I was positive but asymptomatic and that really shifted the fear I was living in so much around getting Covid." Client reports "I'm realizing more and more it's not about me. I spent so much time grieving and he got to just move on" client speaking to shifting patterns she is becoming aware of. Client reports "I have conflicting parts with so many conflicting thoughts" client speaking to what happens inside of her in response to relationship dynamics.

Objective Content

Therapist help space for client to check in. Client shared feeling the heaviness of the political and social events that are occurring in the world. Therapist listened as client shared, offering emotional validation. Therapist and client then moved into going over and updating clients treatment plan to reflect the progress she's made over the last six months as well as the places she continues to get stuck and need support. Therapist then went back into offering space for client to share what is distressing her. Client spoke to multiple areas of her life that she's been tracking. Client is able to recognize her patterning with more awareness however, client states still getting induced by "the negative thoughts and stuck in rumination in spiraling." Therapist asked open-ended questions, offered supportive reflections, emotional validation, reframes and psychoeducation. Client engaged with interventions used. Therapist gave client homework to track one of her patterns. Client expressed willingness to engage with homework outside of session and will report back what she notices.

Interventions Used

The following interventions were used: Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Cognitive Challenging, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Review of Treatment Plan/Progress, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, Grounding/ resourcing, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease "anxiety, panic, and stress" connected to MS diagnosis. Client will decrease her distress level from a 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 9/11/2025 9:02 AM - 10:05 AM

Duration: 63 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

-
2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her NS in the moment. Client will be tracking her triggers to be able to resource and ground herself when triggered from a distress level of 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

3. Client will explore and use cognitive reframing around "obsessive/ ruminating thought" patterns that are currently at distress level of 10. Client will increase her awareness and ability to challenge her negative thought patterns and offer reframes to decrease distressing thought patterns from a level 10 to 9 over the next six months aeb client report in session.

Progress: Progressing

4. Therapist will provide client with PRI, attachment, parts work, and nervous system regulation psychoeducation and tools to use to increase client's awareness of the trigger's in her system and decrease client's "anxiety, overthinking, and self-consciousness" connected to interpersonal relationship dynamics. Client will decrease her distress level from a level 6 to a level 5 over the next 6 months aeb client report in session.

Progress: Progressing

5. Client reports "being told by doctors the physical manifestation of stress can have a negative impact on client's well being has led to an increase in "fear and worry" around MS diagnosis. Client will continue to learn and implement stress reduction tools and resources to decrease the "fear and worry" from a distress level of 10 to a 9 over the next six months aeb client report in session.

Progress: Progressing

6. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was at home for telehealth session.

Plan

Therapist and client will continue exploring client's patterning, grounding and resourcing her anxiety, and figure out ways to meet her needs inter personally and intra personally. As well as explore client's parts using parts work. Therapist will hold space for client to continue to explore and speak to client's experience with MS. Along with building client's capacity for internal resourcing to be less reliant on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 9/11/2025 9:02 AM - 10:05 AM

Duration: 63 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 9/12/2025 at 9:17 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 9/13/2025 at 9:33 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 9/18/2025 9:03 AM - 9:57 AM

Duration: 54 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Euthymic

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 9/18/2025 9:03 AM - 9:57 AM

Duration: 54 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "Wellbutrin 300 mgs for anxiety PCP Christa Toomre is the prescribing physician." Client reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." Client reports "I also take a ton of Vitamins." Client reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

Client reports "I'm upset and OK at the same time. I'm done and I feel good and confident about that" client speaking to the relationship dynamics with her ex. Client reports "there was a lot of fighting and I tried to make a conscious effort to pay attention to the dynamics while I was home. I come home and it's like he's already on edge" client sharing her experience and what she notices about the dynamics between her and her father. Client reports "I feel guilty, horrible, and disappointed with myself. There is resentment and embarrassment" client acknowledging the emotional impact of the relationship dynamics with her father.

Objective Content

Therapist held space for client to check in. Client shared how she was feeling and updates on relationship dynamics with her ex. Therapist listened as client shared. Therapist and client explored the relationship dynamics with her ex and the impact those dynamics had on her over this last week. Client shared her emotional experience and how she responded to her ex's behavior. Client also stated feeling "stressed about work." Therapist offered supportive reflections, emotional validation, and asked open-ended questions. Therapist and client then moved into exploring clients relationship with her father and the dynamics that arise between the two of them. Client stated the impact the dynamics between her and her father have on her. Client named feeling "confused" as to why they keep finding themselves in the same pattern. Client also acknowledged other relationships where the same dynamic shows up between her and her dad. Therapist and client then explored how the pattern and other relationship dynamics are interconnected. Along with the attachment wounding at play. Therapist offered observations, psych-ed, reframes, and grounding and resourcing tools to work with clients emotional experience. Therapist also offered ways for the client to explore for herself what is at the root of the dynamics between her and her father. Therapist and client also explored the attachment wounds at play using parts work. Client was open and engaged with interventions used and expressed openness and willingness to explore for herself what is happening for her within the dynamics and will report back what she notices.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Parts work, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 9/18/2025 9:03 AM - 9:57 AM

Duration: 54 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

-
1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease "anxiety, panic, and stress" connected to MS diagnosis. Client will decrease her distress level from a 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her NS in the moment. Client will be tracking her triggers to be able to resource and ground herself when triggered from a distress level of 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

3. Client will explore and use cognitive reframing around "obsessive/ ruminating thought" patterns that are currently at distress level of 10. Client will increase her awareness and ability to challenge her negative thought patterns and offer reframes to decrease distressing thought patterns from a level 10 to 9 over the next six months aeb client report in session.

Progress: Progressing

4. Therapist will provide client with PRI, attachment, parts work, and nervous system regulation psychoeducation and tools to use to increase client's awareness of the trigger's in her system and decrease client's "anxiety, overthinking, and self-consciousness" connected to interpersonal relationship dynamics. Client will decrease her distress level from a level 6 to a level 5 over the next 6 months aeb client report in session.

Progress: Progressing

5. Client reports "being told by doctors the physical manifestation of stress can have a negative impact on client's well being has led to an increase in "fear and worry" around MS diagnosis. Client will continue to learn and implement stress reduction tools and resources to decrease the "fear and worry" from a distress level of 10 to a 9 over the next six months aeb client report in session.

Progress: Progressing

6. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was at home for telehealth session.

Plan

Therapist and client will continue exploring client's patterning interpersonally and intrapersonally, practice grounding and resourcing her anxiety, overthinking, and self-consciousness and work with her attachment wounding in relationship. Along with exploring client's younger

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 9/18/2025 9:03 AM - 9:57 AM

Duration: 54 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

parts using parts work. Therapist will hold space for client to continue to explore and speak to client's experience with MS. Along with building client's capacity for internal resourcing to be less reliant on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 9/18/2025 at 2:59 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 9/18/2025 at 8:48 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 9/23/2025 8:47 AM - 9:46 AM

Duration: 59 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Euthymic

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 9/23/2025 8:47 AM - 9:46 AM

Duration: 59 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "Wellbutrin 300 mgs for anxiety PCP Christa Toomre is the prescribing physician." Client reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." Client reports "I also take a ton of Vitamins." Client reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

Client reports "I've been in my head about the story he tells other people. I get roped into this type of thinking it feels like a control piece and I don't want to get caught off guard" client sharing what she is currently noticing about how she is relating to her break up, the younger parts that are arising, and the motivation for the coping mechanisms she is engaging with. Client reports "I want to get better at understanding myself." Client asked questions for clarification around the modalities therapist uses. Client reports "I am resistant to building community in the MS world and letting MS be a big part of my life" client sharing where she is currently at in regard to engaging with community in the MS world as she is preparing for a trip with the MS community.

Objective Content

Therapist held space for client to check in. Client shared how she was feeling and named where she was currently at in regard to the end of her romantic relationship. Therapist listened to his client shared. Therapist asked client questions regarding statement client was making about where she is at regarding the ending of her romantic relationship. Client engaged with questions and shared what was coming up for her. Client then was curious about modalities and interventions therapist uses. Client wanted a refresher because there's a part of her that feels "like she does therapy wrong." Therapist offered client a breakdown of the psych-ed modalities, tools, and resources offered in session and why therapist works from those frameworks. Client listened and continue to ask questions as therapist offered responses to clients questions. Therapist also shared with client that she is not doing therapy wrong and informed the client we can continue to look at the mechanisms that show up and inform how she shows up in therapy and any connections that has to how she shows up in her daily life. Client was open and receptive to therapist offerings and expressed curiosity, resonance, and understanding for the modalities and tools used. Client then spoke to an upcoming trip she's going on for her MS. Therapist and client explored the trip, the emotional experience client is having in relation to the trip, and the "resistance" that arises for the client around "building community or identifying with MS." Therapist explored clients resistance with her using Parts work, offering supportive reflections, emotional validation, and continuing to ask open-ended questions. Throughout session client engaged with interventions used and expressed "appreciation for all that the therapist offered" in session today.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Parts work, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 9/23/2025 8:47 AM - 9:46 AM

Duration: 59 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease "anxiety, panic, and stress" connected to MS diagnosis. Client will decrease her distress level from a 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her NS in the moment. Client will be tracking her triggers to be able to resource and ground herself when triggered from a distress level of 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

3. Client will explore and use cognitive reframing around "obsessive/ ruminating thought" patterns that are currently at distress level of 10. Client will increase her awareness and ability to challenge her negative thought patterns and offer reframes to decrease distressing thought patterns from a level 10 to 9 over the next six months aeb client report in session.

Progress: Progressing

4. Therapist will provide client with PRI, attachment, parts work, and nervous system regulation psychoeducation and tools to use to increase client's awareness of the trigger's in her system and decrease client's "anxiety, overthinking, and self-consciousness" connected to interpersonal relationship dynamics. Client will decrease her distress level from a level 6 to a level 5 over the next 6 months aeb client report in session.

Progress: Progressing

5. Client reports "being told by doctors the physical manifestation of stress can have a negative impact on client's well being has led to an increase in "fear and worry" around MS diagnosis. Client will continue to learn and implement stress reduction tools and resources to decrease the "fear and worry" from a distress level of 10 to a 9 over the next six months aeb client report in session.

Progress: Progressing

6. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was at home for telehealth session.

Plan

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 9/23/2025 8:47 AM - 9:46 AM

Duration: 59 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Therapist and client will continue exploring client's patterning interpersonally and intrapersonally, practice grounding and resourcing her anxiety, overthinking, and self-consciousness and work with her attachment wounding in relationship. Along with exploring client's younger parts using parts work. Therapist will hold space for client to continue to explore and speak to client's experience with MS. Along with building client's capacity for internal resourcing to be less reliant on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 9/29/2025 at 5:49 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 9/30/2025 at 3:30 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 10/2/2025 9:02 AM - 9:56 AM

Duration: 54 minutes

Service Code: 90837

Location: Teletherapy- Patient is Other Than Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Euthymic

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 10/2/2025 9:02 AM - 9:56 AM

Duration: 54 minutes

Service Code: 90837

Location: Teletherapy- Patient is Other Than Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "Wellbutrin 300 mgs for anxiety PCP Christa Toomre is the prescribing physician." clt. reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." clt. reports "I also take a ton of Vitamins." clt. reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

clt. reports "I felt good and sure for the last month and then noticed my self regressing after our interaction" clt. shared the impact of the interaction with her ex.

clt. reports "there's a lack of trust in there motivation and intent" clt. sharing how she is relating to the relationship patterns explored in session.

clt. reports "I felt regretful looking back and tried to not go tit for tat" what clt. noticed about her patterning and coping strategies in relationship.

Objective Content

th. and clt. explored clt's. relationship patterns. clt. shared an interaction she had with her ex. clt. stated noticing how her patterns are showing up in her relationships with more awareness and curiosity. clt. named a pattern that is no longer serving her. th. offered reframes, responded to clt. questions, and asked open ended questions. th. and clt. explored clt's. emotions. clt. shared feeling "embarrassment, guilt, and shame." clt. is able to name her emotional experience and notices where she continues to get stuck in her coping strategies to mitigate emotional pain and discomfort. th. offered supportive reflections and emotional validation. th. and clt. explored clt's. coping strategies. clt. expressed paying attention to her pattern of "cycling through the interactions" client names getting "stuck in rumination around how conversations could have gone." th. and clt. are working on implementing new coping strategies through psych-ed, grounding and resourcing tools, and Parts work exercises. clt. engaged with interventions used throughout session.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Parts work, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease "anxiety, panic, and stress" connected to MS diagnosis. Client will decrease her distress level from a 8 to a level 7 over the next six months aeb client report in session.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 10/2/2025 9:02 AM - 9:56 AM

Duration: 54 minutes

Service Code: 90837

Location: Teletherapy- Patient is Other Than Home

Participants: Client Only

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her NS in the moment. Client will be tracking her triggers to be able to resource and ground herself when triggered from a distress level of 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

3. Client will explore and use cognitive reframing around "obsessive/ ruminating thought" patterns that are currently at distress level of 10. Client will increase her awareness and ability to challenge her negative thought patterns and offer reframes to decrease distressing thought patterns from a level 10 to 9 over the next six months aeb client report in session.

Progress: Progressing

4. Therapist will provide client with PRI, attachment, parts work, and nervous system regulation psychoeducation and tools to use to increase client's awareness of the trigger's in her system and decrease client's "anxiety, overthinking, and self-consciousness" connected to interpersonal relationship dynamics. Client will decrease her distress level from a level 6 to a level 5 over the next 6 months aeb client report in session.

Progress: Progressing

5. Client reports "being told by doctors the physical manifestation of stress can have a negative impact on client's well being has led to an increase in "fear and worry" around MS diagnosis. Client will continue to learn and implement stress reduction tools and resources to decrease the "fear and worry" from a distress level of 10 to a 9 over the next six months aeb client report in session.

Progress: Progressing

6. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

clt. was pet sitting for telehealth session.

Plan

th. and clt. will continue exploring clt's. patterning interpersonally and intrapersonally, practice grounding and resourcing her anxiety, overthinking, and self-consciousness and work with her attachment wounding in relationship. Along with exploring clt's. younger Parts using parts work. th. will hold space for clt. to continue to explore and speak to clt's. experience with MS. Along with building clt's. capacity for internal resourcing to be less reliant on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 10/2/2025 9:02 AM - 9:56 AM

Duration: 54 minutes

Service Code: 90837

Location: Teletherapy- Patient is Other Than Home

Participants: Client Only

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 10/5/2025 at 8:24 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 10/7/2025 at 3:43 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 10/17/2025 12:01 PM - 1:00 PM

Duration: 59 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 10/17/2025 12:01 PM - 1:00 PM

Duration: 59 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "Wellbutrin 300 mgs for anxiety PCP Christa Toomre is the prescribing physician." clt. reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." clt. reports "I also take a ton of Vitamins." clt. reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

clt. reports "I had a relapse in my boundary with my ex. I was going to be self-aggressive, but caught myself doing that and recognized that the stories and beliefs were not true" clt. sharing about why the interaction occurred from her perspective, as well as, the self-aggressive behavior she noticed arise and the choice to not buy into those stories and beliefs.

clt. reports "I felt very young. I wanted to be chosen and not forgotten. I attach the relationship to my worth" clt. sharing the beliefs and needs connected to her attachment wounding.

clt. reports "I want to take some much responsibility for his needs and emotions. I have a high expectation in relationship and have a belief that I know what people need and what would be best for them" clt. continuing to share why she thinks a pattern of hers continues to show up in her relationships.

Objective Content

clt. started by sharing how she was feeling and then moved into sharing an interaction that occurred in a relationship of hers. clt. shared recognizing a pattern of hers that she is continuing to bring more awareness to connected her attachment patterning and the coping strategies that come from feeling "abandoned" and/or "rejected." th. listened as clt. shared tracking themes and patterns, and offering emotional validation and supportive reflections. th. and clt. explored clt's. attachment pattern and the coping strategies the clt. was speaking to. clt. continued to share the beliefs, stories, and emotions connected to feeling "rejected." Therapist asked open-ended questions, offered observations, and psych-ed on states of nervous system activation and attachment patterning. th. and clt. explored the parts of the clt. she moves from when her nervous system becomes activated and anxious. clt. shared that she could see she was moving from a younger part of herself and the beliefs and stories the younger parts of her hold. clt. also named the emotional and somatic response that showed up in her body. th. used Parts work to explore clt's. younger parts and offered grounding and resourcing tools for clt. to implement to work with regulating her nervous system and the younger parts that showed up within the relationship dynamic. clt. engaged with the interventions used and expressed resonance and understanding for psych-ed offered. th. gave clt. HW to track and learn about her attachment patten. clt. expressed interest in HW and stated she would report back what she learns and notices when working with this pattern outside of session.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Parts work, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 10/17/2025 12:01 PM - 1:00 PM

Duration: 59 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease "anxiety, panic, and stress" connected to MS diagnosis. Client will decrease her distress level from a 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her NS in the moment. Client will be tracking her triggers to be able to resource and ground herself when triggered from a distress level of 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

3. Client will explore and use cognitive reframing around "obsessive/ ruminating thought" patterns that are currently at distress level of 10. Client will increase her awareness and ability to challenge her negative thought patterns and offer reframes to decrease distressing thought patterns from a level 10 to 9 over the next six months aeb client report in session.

Progress: Progressing

4. Therapist will provide client with PRI, attachment, parts work, and nervous system regulation psychoeducation and tools to use to increase client's awareness of the trigger's in her system and decrease client's "anxiety, overthinking, and self-consciousness" connected to interpersonal relationship dynamics. Client will decrease her distress level from a level 6 to a level 5 over the next 6 months aeb client report in session.

Progress: Progressing

5. Client reports "being told by doctors the physical manifestation of stress can have a negative impact on client's well being has led to an increase in "fear and worry" around MS diagnosis. Client will continue to learn and implement stress reduction tools and resources to decrease the "fear and worry" from a distress level of 10 to a 9 over the next six months aeb client report in session.

Progress: Progressing

6. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

clt. was at home for telehealth session.

Plan

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 10/17/2025 12:01 PM - 1:00 PM

Duration: 59 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

th. and clt. will continue exploring clt's. patterning interpersonally and intrapersonally, practice grounding and resourcing her anxiety, overthinking, and self-consciousness and work with her attachment patterning in relationship. Along with exploring clt's. younger Parts using parts work. th. will hold space for clt. to continue to explore and speak to clt's. experience with MS. Along with building clt's. capacity for internal resourcing to be less reliant on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 10/19/2025 at 3:49 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 10/20/2025 at 6:20 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 10/23/2025 9:02 AM - 10:00 AM

Duration: 58 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Euthymic

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 10/23/2025 9:02 AM - 10:00 AM

Duration: 58 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "Wellbutrin 300 mgs for anxiety PCP Christa Toomre is the prescribing physician." clt. reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." clt. reports "I also take a ton of Vitamins." clt. reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

clt. reports "I have expectations of others and myself and I believe I know what's best for people" clt. sharing the beliefs and a pattern that shows up in her relationships that hinders her ability for connection because of the impact it has on her relationships.

clt. reports "it leads to disappointment and negative self-talk" clt. sharing the negative impact of her patterning around having high expectations of herself and others.

clt. reports "I feel in control and safe" clt. sharing why having high standards is a value of hers.

Objective Content

clt. started by sharing how she was feeling. th. and clt. continued to explore clt's. patterning and how she relates to her patterning. clt. shared the beliefs she holds about others and herself connected to "expectations and comparison." th. asked open ended questions, offered supportive reflections, and emotional validation. clt. made a statement about two of her patterns and then asked th. clarifying question. th. inquired with clt. regarding her statement before offering psych-ed and continuing to ask clt. questions regarding the statement she made. th. and clt. explored clt's. coping strategies. clt. was able to see how her "high expectations and perfectionism" are forms of "creating control and safety." th. offered psych-ed, reframes, and grounding and resourcing tools for clt. to use with regulating her NS. th. gave clt. HW to track what happens mentally, physically, and emotionally when she is triggered to continue to bring awareness to clt's. patterning and coping strategies. clt. expressed openness to tracking her experience and reporting back what she notices so we can continue to work with re patterning her patterns and coping strategies and building clt's. capacity for emotional dysregulation and regulation.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Parts work, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease "anxiety, panic, and stress" connected to MS diagnosis. Client will decrease her distress level from a 8 to a level 7 over the

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 10/23/2025 9:02 AM - 10:00 AM

Duration: 58 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

next six months aeb client report in session.

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her NS in the moment. Client will be tracking her triggers to be able to resource and ground herself when triggered from a distress level of 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

3. Client will explore and use cognitive reframing around "obsessive/ ruminating thought" patterns that are currently at distress level of 10. Client will increase her awareness and ability to challenge her negative thought patterns and offer reframes to decrease distressing thought patterns from a level 10 to 9 over the next six months aeb client report in session.

Progress: Progressing

4. Therapist will provide client with PRI, attachment, parts work, and nervous system regulation psychoeducation and tools to use to increase client's awareness of the trigger's in her system and decrease client's "anxiety, overthinking, and self-consciousness" connected to interpersonal relationship dynamics. Client will decrease her distress level from a level 6 to a level 5 over the next 6 months aeb client report in session.

Progress: Progressing

5. Client reports "being told by doctors the physical manifestation of stress can have a negative impact on client's well being has led to an increase in "fear and worry" around MS diagnosis. Client will continue to learn and implement stress reduction tools and resources to decrease the "fear and worry" from a distress level of 10 to a 9 over the next six months aeb client report in session.

Progress: Progressing

6. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

clt. was at home for telehealth session.

Plan

th. and clt. will continue exploring clt's. patterning interpersonally and intrapersonally, practice grounding and resourcing her anxiety, overthinking, and self-consciousness and work with her attachment patterning in relationship. Along with exploring clt's. younger Parts using parts work. th. will hold space for clt. to continue to explore and speak to clt's. experience with MS. Along with building clt's. capacity for internal resourcing to be less reliant on external resourcing and validation.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 10/23/2025 9:02 AM - 10:00 AM

Duration: 58 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 10/23/2025 at 4:30 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 10/24/2025 at 7:42 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 10/30/2025 9:00 AM - 9:56 AM

Duration: 56 minutes

Service Code: 90837

Location: Teletherapy- Patient is Other Than Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 10/30/2025 9:00 AM - 9:56 AM

Duration: 56 minutes

Service Code: 90837

Location: Teletherapy- Patient is Other Than Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "Wellbutrin 300 mg for anxiety PCP Christa Toomre is the prescribing physician." clt. reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." clt. reports "I also take a ton of Vitamins." clt. reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

clt. reports "I think my self-worth is tied to seeking validation."

clt. reports "I got triggered while I was there and I am feeling guilty about the past interaction" clt. speaking to the emotional impact of recognizing the pattern she was moving from in recent interaction she spoke to that brought relational pattern to light.

clt. reports "I want to be someones first choice and I can see a connection between that and my adoption" clt. speaking to where this belief/ need comes from in childhood.

Objective Content

clt. started by sharing how she was feeling and then spoke to relational patterns and dynamics she is noticing about herself. clt. stated "I notice I am constantly seeking external validation." th. listened as clt. shared. th. and clt. explored clt's. constant seeking of external validation. clt. shared a recent experience that brought this pattern to light in a new way. clt. stated the impact of noticing this pattern and a desire to not move from this pattern in the future. th. offered supportive reflections, emotional validation, and asked open ended questions. th. and clt. explored any childhood roots to this pattern and why this pattern arose. clt. shared from her understanding how this pattern is rooted in childhood. clt. stated "it feels connected to boredom and adoption." th. continued to ask open ended questions and offered psych-ed, reframes, and grounding and resourcing tools. clt. engaged with interventions used. th. gave clt. HW to continue to track what she notices about this pattern as she continues to bring awareness to it. clt. expressed openness to continue to track and bring awareness to the pattern of seeking external validation and report back what she notices.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Parts work, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease "anxiety, panic, and stress" connected to MS diagnosis. Client will decrease her distress level from a 8 to a level 7 over the next six months aeb client report in session.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 10/30/2025 9:00 AM - 9:56 AM

Duration: 56 minutes

Service Code: 90837

Location: Teletherapy- Patient is Other Than Home

Participants: Client Only

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her NS in the moment. Client will be tracking her triggers to be able to resource and ground herself when triggered from a distress level of 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

3. Client will explore and use cognitive reframing around "obsessive/ ruminating thought" patterns that are currently at distress level of 10. Client will increase her awareness and ability to challenge her negative thought patterns and offer reframes to decrease distressing thought patterns from a level 10 to 9 over the next six months aeb client report in session.

Progress: Progressing

4. Therapist will provide client with PRI, attachment, parts work, and nervous system regulation psychoeducation and tools to use to increase client's awareness of the trigger's in her system and decrease client's "anxiety, overthinking, and self-consciousness" connected to interpersonal relationship dynamics. Client will decrease her distress level from a level 6 to a level 5 over the next 6 months aeb client report in session.

Progress: Progressing

5. Client reports "being told by doctors the physical manifestation of stress can have a negative impact on client's well being has led to an increase in "fear and worry" around MS diagnosis. Client will continue to learn and implement stress reduction tools and resources to decrease the "fear and worry" from a distress level of 10 to a 9 over the next six months aeb client report in session.

Progress: Progressing

6. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

clt. was in her car for telehealth session.

Plan

th. and clt. will continue exploring clt's. patterning interpersonally and intrapersonally, practice grounding and resourcing her anxiety, overthinking, and self-consciousness and work with her attachment patterning in relationship. Along with exploring clt's. younger Parts using parts work. th. will hold space for clt. to continue to explore and speak to clt's. experience with MS. Along with building clt's. capacity for internal resourcing to be less reliant on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 10/30/2025 9:00 AM - 9:56 AM

Duration: 56 minutes

Service Code: 90837

Location: Teletherapy- Patient is Other Than Home

Participants: Client Only

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 11/6/2025 at 9:46 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 11/6/2025 at 4:22 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 11/4/2025 10:00 AM - 10:58 AM

Duration: 58 minutes

Service Code: 90837

Location: Teletherapy- Patient is Other Than Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 11/4/2025 10:00 AM - 10:58 AM

Duration: 58 minutes

Service Code: 90837

Location: Teletherapy- Patient is Other Than Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "Wellbutrin 300 mg for anxiety PCP Christa Toomre is the prescribing physician." clt. reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." clt. reports "I also take a ton of Vitamins." clt. reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

clt. reports "I have an ovarian cyst that I need to get looked at again. I had surgery in the past to get one removed and it was the size of a cantaloupe" clt. sharing about her physical health issues.
clt. reports "I was worried about feeling left out and the dynamics changing. I view change as a negative thing" clt. sharing younger beliefs and fears that came up with a new relationship dynamic shift.
clt. reports "I felt really triggered by it and part of me felt jealous and another part felt agitated" clt. sharing about how she noticed her system respond to relationship dynamics shifting.

Objective Content

clt. started by sharing how she was feeling and updates on physical health related issues. th. listened as clt. shared. clt. then spoke to a coping strategy she notices show up in her relationships. th. and clt. explored clt's. coping strategies in her relationships. clt. engaged in exploration and stated "I always place blame" clt. recognizes this coping strategy is not supportive of her relationships with others and herself. th. asked open ended questions, offered supportive reflections, reframes, emotional validation, and psych-ed. clt. then spoke to a relationship interaction where she noticed she became activated. th. and clt. explored clt's. relationship patterns, emotions, and activation. clt. shared childhood memories and relationship dynamics that were connected to her present day activation. clt. stated feeling "worried I was going to be left out." th. continued to offer psych-ed, reframes, and ask open ended questions. th. also offered grounding and resourcing tools for clt. to practice to work with decreasing her NS response. As well as, ways to work with shifting and reframing her belief systems and stories. clt. engaged with interventions used.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Parts work, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease "anxiety, panic, and stress" connected to MS diagnosis. Client will decrease her distress level from a 8 to a level 7 over the

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 11/4/2025 10:00 AM - 10:58 AM

Duration: 58 minutes

Service Code: 90837

Location: Teletherapy- Patient is Other Than Home

Participants: Client Only

next six months aeb client report in session.

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her NS in the moment. Client will be tracking her triggers to be able to resource and ground herself when triggered from a distress level of 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

3. Client will explore and use cognitive reframing around "obsessive/ ruminating thought" patterns that are currently at distress level of 10. Client will increase her awareness and ability to challenge her negative thought patterns and offer reframes to decrease distressing thought patterns from a level 10 to 9 over the next six months aeb client report in session.

Progress: Progressing

4. Therapist will provide client with PRI, attachment, parts work, and nervous system regulation psychoeducation and tools to use to increase client's awareness of the trigger's in her system and decrease client's "anxiety, overthinking, and self-consciousness" connected to interpersonal relationship dynamics. Client will decrease her distress level from a level 6 to a level 5 over the next 6 months aeb client report in session.

Progress: Progressing

5. Client reports "being told by doctors the physical manifestation of stress can have a negative impact on client's well being has led to an increase in "fear and worry" around MS diagnosis. Client will continue to learn and implement stress reduction tools and resources to decrease the "fear and worry" from a distress level of 10 to a 9 over the next six months aeb client report in session.

Progress: Progressing

6. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

clt. was in her car for telehealth session.

Plan

th. and clt. will continue exploring clt's. patterning interpersonally and intrapersonally, practice grounding and resourcing her anxiety, overthinking, and self-consciousness and work with her attachment patterning in relationship. Along with exploring clt's. younger Parts using parts work. th. will hold space for clt. to continue to explore and speak to clt's. experience with MS. Along with building clt's. capacity for internal resourcing to be less reliant on external resourcing and validation.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 11/4/2025 10:00 AM - 10:58 AM

Duration: 58 minutes

Service Code: 90837

Location: Teletherapy- Patient is Other Than Home

Participants: Client Only

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 11/6/2025 at 10:38 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 11/6/2025 at 4:25 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 11/13/2025 9:02 AM - 9:57 AM

Duration: 55 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 11/13/2025 9:02 AM - 9:57 AM

Duration: 55 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "Wellbutrin 300 mg for anxiety PCP Christa Toomre is the prescribing physician." clt. reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." clt. reports "I also take a ton of Vitamins." clt. reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

clt. reports "I was getting upset with myself and asking did I let fear control my decision" clt. sharing her fear about having the fear arise. clt. reports "there is a fear of dying connected to my parents losing their son and my health anxiety connected to my MS diagnosis" clt. sharing the past and present fears connected to the fear of dying. clt. reports "it was ingrained in me to be afraid of driving and I don't want to put my parents through the same thing that happened with my brother" clt. sharing aspects that are motivating and rooted in the fear she was speaking to.

Objective Content

clt. started by sharing how she was feeling and named an experience she had since the last session that activated a fear response in her system. th. listened as clt. shared, offering supportive reflections and emotional validation. th. and clt. explored clt's. fear. clt. shared how the fear showed up in her brain and body and then stated "there is a fear about the fear." clt. was able to make connections to the present day fear and past traumatic events that occurred that were driving the present day fear. th. offered psych-ed, reframes, emotional validation, and asked open ended questions. clt. engaged with interventions used. th. and clt. practiced somatic exercises to work with the present and past fears. th. modeled and explained the exercises to the clt. clt. participated in the exercises and named what she noticed happen during the exercises and stated feeling "more relaxed and the pit in my stomach went away" afterwards.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Experiential Somatic Exercise, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Guided Imagery, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Parts work, Preventative Services, Psycho-Education, Relaxation/Deep Breathing, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease "anxiety, panic, and stress" connected to MS diagnosis. Client will decrease her distress level from a 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 11/13/2025 9:02 AM - 9:57 AM

Duration: 55 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

-
2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her NS in the moment. Client will be tracking her triggers to be able to resource and ground herself when triggered from a distress level of 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

3. Client will explore and use cognitive reframing around "obsessive/ ruminating thought" patterns that are currently at distress level of 10. Client will increase her awareness and ability to challenge her negative thought patterns and offer reframes to decrease distressing thought patterns from a level 10 to 9 over the next six months aeb client report in session.

Progress: Progressing

4. Therapist will provide client with PRI, attachment, parts work, and nervous system regulation psychoeducation and tools to use to increase client's awareness of the trigger's in her system and decrease client's "anxiety, overthinking, and self-consciousness" connected to interpersonal relationship dynamics. Client will decrease her distress level from a level 6 to a level 5 over the next 6 months aeb client report in session.

Progress: Progressing

5. Client reports "being told by doctors the physical manifestation of stress can have a negative impact on client's well being has led to an increase in "fear and worry" around MS diagnosis. Client will continue to learn and implement stress reduction tools and resources to decrease the "fear and worry" from a distress level of 10 to a 9 over the next six months aeb client report in session.

Progress: Progressing

6. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

clt. was at home for telehealth session.

Plan

th. and clt. will continue exploring clt's. patterning interpersonally and intrapersonally, practice grounding and resourcing her anxiety, overthinking, and self-consciousness and work with her attachment patterning in relationship. Along with exploring clt's. younger Parts using parts work. th. will hold space for clt. to continue to explore and speak to clt's. experience with MS. Along with building clt's. capacity for internal resourcing to be less reliant on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 11/13/2025 9:02 AM - 9:57 AM

Duration: 55 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 11/13/2025 at 10:15 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 11/14/2025 at 5:34 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 11/20/2025 9:02 AM - 10:02 AM

Duration: 60 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 11/20/2025 9:02 AM - 10:02 AM

Duration: 60 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "Wellbutrin 300 mg for anxiety PCP Christa Toomre is the prescribing physician." clt. reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." clt. reports "I also take a ton of Vitamins." clt. reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

clt. reports "when I get triggered into comparison I start mapping out myself against others" clt. sharing about what happens mentally when she is triggered physically and emotionally by the belief "I don't have what others have."

clt. reports "I have been feeling on edge and getting mad at myself for having these comparison thoughts occur" clt. sharing the impact and how she is relating to herself when her psychological defense beliefs and stories arise.

clt. reports "I feel confused all of the time" clt. speaking to a relationship dynamic in which this pattern is currently being played out. clt. is working on leaving this dynamic and continues to feel and be stuck in the dynamic due to her NS response.

Objective Content

clt. started by sharing how she was feeling and stated feeling a "sadness due to the holidays." th. listened as clt. shared. th. and clt. explored clt's. emotions focusing on her sadness. clt. shared the wishing defenses that are arising and the patterns of "comparison" from feeling "disappointment" are arising. th. asked open ended questions, offered supportive reflections, and emotional validation. th. and clt. explored clt's. psychological defenses of "wishing and comparison." clt. shared the stories and beliefs that come up for her connected to her emotions that illicit her defenses. clt. stated "I am scared of being left behind." clt. also made connections between these emotions and beliefs and why she goes into "believing I know what's best or try to boss or control the situation." While clt. is becoming more aware of her patterning when her NS becomes activated she is noticing she gets pulled into those stories and beliefs and starts to move from her protectors and defense structures. th. offered psych-ed, reframes, observations, and grounding and resourcing tools for clt. to practice to work with decreasing her psychological defense structures and building her capacity to be with "disappointment, sadness, and fear." clt. engaged with interventions used throughout session, took notes on psych-ed and resources offered, and expressed appreciation for th. support.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Parts work, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 11/20/2025 9:02 AM - 10:02 AM

Duration: 60 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

-
1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease "anxiety, panic, and stress" connected to MS diagnosis. Client will decrease her distress level from a 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her NS in the moment. Client will be tracking her triggers to be able to resource and ground herself when triggered from a distress level of 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

3. Client will explore and use cognitive reframing around "obsessive/ ruminating thought" patterns that are currently at distress level of 10. Client will increase her awareness and ability to challenge her negative thought patterns and offer reframes to decrease distressing thought patterns from a level 10 to 9 over the next six months aeb client report in session.

Progress: Progressing

4. Therapist will provide client with PRI, attachment, parts work, and nervous system regulation psychoeducation and tools to use to increase client's awareness of the trigger's in her system and decrease client's "anxiety, overthinking, and self-consciousness" connected to interpersonal relationship dynamics. Client will decrease her distress level from a level 6 to a level 5 over the next 6 months aeb client report in session.

Progress: Progressing

5. Client reports "being told by doctors the physical manifestation of stress can have a negative impact on client's well being has led to an increase in "fear and worry" around MS diagnosis. Client will continue to learn and implement stress reduction tools and resources to decrease the "fear and worry" from a distress level of 10 to a 9 over the next six months aeb client report in session.

Progress: Progressing

6. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

clt. was at home for telehealth session.

Plan

th. and clt. will continue exploring clt's. patterning interpersonally and intrapersonally, practice grounding and resourcing her "anxiety, overthinking, and self-consciousness" and work with her attachment patterning in relationships. Along with exploring clt's. psychological

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 11/20/2025 9:02 AM - 10:02 AM

Duration: 60 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

defense structures using PRI psych-ed, Parts work, and somatic exercises. th. will hold space for clt. to continue to explore and speak to clt's. experience with MS. Along with building clt's. capacity for increased internal resourcing to decrease clt's. reliance on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 11/20/2025 at 10:19 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 11/20/2025 at 1:03 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 12/4/2025 9:02 AM - 10:09 AM

Duration: 67 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious and Depressed

Affect: Congruent

Insight: Good

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 12/4/2025 9:02 AM - 10:09 AM

Duration: 67 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "Wellbutrin 300 mg for anxiety PCP Christa Toomre is the prescribing physician." clt. reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." clt. reports "I also take a ton of Vitamins." clt. reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

clt. reports "I can't get out of this toxic cycle. It's so confusing and I feel distressed all the time" clt. speaking to the relationship dynamic she was speaking too.

clt. reports feeling "overwhelmed, sad, and confused" clt. sharing the emotions that were present during session.

clt. reports "I've been in anxious thought loops and spirals" regarding being stuck in the pattern she was speaking to.

Objective Content

clt. came into session visibly distressed. clt. shared feeling "disappointed and embarrassed" that she is still stuck in a cycle she is working hard to get out of in romantic relationships. clt. expressed the pattern is "so distressing" to her. th. listened as clt. shared, offering emotional validation and holding space for clt. to emotionally express her sadness connected to her "disappointment and embarrassment." th. and clt. explored clt's. emotions. clt. shared the stories and beliefs connected to her emotions. clt. also shared her "fears" and what she notices about what is keeping this pattern in place. th. offered supportive reflections, psych-ed, and asked open ended questions. th. and clt. explored clt's. relationship dynamics. clt. shared the pattern that her system gets looped into in romantic relationships and expressed "confusion around why it's so hard to let this relationship go." th. continued to ask open ended questions and offer psych-ed. th. also offered reframes and grounding and resourcing tools for clt. practice when her NS is activated regarding the dynamic the clt. is struggling to break. th. invited clt. to practice the NS exercise to work resourcing herself out of her distress. clt. participated in the exercises and stated feeling "more chill" meaning more leveled out afterward the exercises and by the end of session.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Nervous System Exercise, Open Ended Questioning, Parts work, Preventative Services, Psycho-Education, Relaxation/Deep Breathing, Somatic Exercises, Structured Problem Solving, Supportive Reflection, Symptom Management, and Validation Exercises.

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease "anxiety, panic, and stress" connected to MS diagnosis. Client will decrease her distress level from a 8 to a level 7 over the next six months aeb client report in session.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 12/4/2025 9:02 AM - 10:09 AM

Duration: 67 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her NS in the moment. Client will be tracking her triggers to be able to resource and ground herself when triggered from a distress level of 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

3. Client will explore and use cognitive reframing around "obsessive/ ruminating thought" patterns that are currently at distress level of 10. Client will increase her awareness and ability to challenge her negative thought patterns and offer reframes to decrease distressing thought patterns from a level 10 to 9 over the next six months aeb client report in session.

Progress: Progressing

4. Therapist will provide client with PRI, attachment, parts work, and nervous system regulation psychoeducation and tools to use to increase client's awareness of the trigger's in her system and decrease client's "anxiety, overthinking, and self-consciousness" connected to interpersonal relationship dynamics. Client will decrease her distress level from a level 6 to a level 5 over the next 6 months aeb client report in session.

Progress: Progressing

5. Client reports "being told by doctors the physical manifestation of stress can have a negative impact on client's well being has led to an increase in "fear and worry" around MS diagnosis. Client will continue to learn and implement stress reduction tools and resources to decrease the "fear and worry" from a distress level of 10 to a 9 over the next six months aeb client report in session.

Progress: Progressing

6. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

clt. was at home for telehealth session.

th. and clt. took extra time to ground and resource clt's. physical, mental, and emotional distress present in today's session.

clt. still experiences high levels of "anxiety, stress, and panic" when triggered by the emotional weight of living with a chronic MS diagnosis; continued therapy is needed to help client continue to work towards her goal of lowering "anxiety, stress and panic" and increasing her knowledge and application of tools and resources.

Plan

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 12/4/2025 9:02 AM - 10:09 AM

Duration: 67 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

th. and clt. will continue exploring clt's. patterning interpersonally and intrapersonally, practice grounding and resourcing her "anxiety, overthinking, and self-consciousness" and work with her attachment patterning in relationships. Along with exploring clt's. psychological defense structures using PRI psych-ed, Parts work, and somatic exercises. th. will hold space for clt. to continue to explore and speak to clt's. experience with MS. Along with building clt's. capacity for increased internal resourcing to decrease clt's. reliance on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 12/7/2025 at 11:45 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 12/9/2025 at 1:41 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 12/11/2025 9:02 AM - 9:57 AM

Duration: 55 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Good

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 12/11/2025 9:02 AM - 9:57 AM

Duration: 55 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "Wellbutrin 300 mg for anxiety PCP Christa Toomre is the prescribing physician." clt. reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." clt. reports "I also take a ton of Vitamins." clt. reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

clt. reports "I feel less emotional than last week but I still haven't made any changes and I don't understand why if I know it doesn't work I can't choose something different" clt. sharing about the relational pattern.

clt. reports "insecurity, pride, and proximity" are factors that keep her stuck in the loop.

clt. reports "I wanna fix him and if I did that it would say I am meaningful, but I also don't want it either and I don't know why I can't just choose that" clt. sharing more about what keeps her stuck in the pattern and the internal conflict around having competing desires, knowings, and needs.

Objective Content

clt. shared how she was feeling and stated "pulling a back muscle." clt. then went back into sharing more about the pattern we worked on in the last session. th. listened as clt. shared, offering emotional validation, supportive reflections, and asking open ended questions. th. and clt. continued to explore clt's. relational pattern. clt. was able to offer some insights as to why she continues to get "stuck and move" from this pattern. clt. also expresses "not understanding" why she is still stuck in this pattern. th. continued to ask open ended questions, offered observations, psych-ed, and reframes. th. and clt. explored clt's. attachment style and how that is connected to the pattern. clt. asked questions and th. provided responses and also invited clt. to see what is occurring internally that would be connected to keeping this pattern in place. clt. shared "lacking self-discipline" and "not wanting to be alone." th. continued to offer psych-ed, emotional validation, supportive reflections, and ask open ended questions. clt. expressed appreciation for th. support. th. also gave clt. HW to practice exercise to work with the emotional distress and mental ideologies connected to the pattern. clt. expressed willingness to practice HW and report back what she notices.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Parts work, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease "anxiety, panic, and stress" connected to MS diagnosis. Client will decrease her distress level from a 8 to a level 7 over the

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 12/11/2025 9:02 AM - 9:57 AM

Duration: 55 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

next six months aeb client report in session.

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her NS in the moment. Client will be tracking her triggers to be able to resource and ground herself when triggered from a distress level of 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

3. Client will explore and use cognitive reframing around "obsessive/ ruminating thought" patterns that are currently at distress level of 10. Client will increase her awareness and ability to challenge her negative thought patterns and offer reframes to decrease distressing thought patterns from a level 10 to 9 over the next six months aeb client report in session.

Progress: Progressing

4. Therapist will provide client with PRI, attachment, parts work, and nervous system regulation psychoeducation and tools to use to increase client's awareness of the trigger's in her system and decrease client's "anxiety, overthinking, and self-consciousness" connected to interpersonal relationship dynamics. Client will decrease her distress level from a level 6 to a level 5 over the next 6 months aeb client report in session.

Progress: Progressing

5. Client reports "being told by doctors the physical manifestation of stress can have a negative impact on client's well being has led to an increase in "fear and worry" around MS diagnosis. Client will continue to learn and implement stress reduction tools and resources to decrease the "fear and worry" from a distress level of 10 to a 9 over the next six months aeb client report in session.

Progress: Progressing

6. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

clt. was at home for telehealth session.

th. and clt. took extra time to ground and resource clt's. physical, mental, and emotional distress present in today's session.

clt. still experiences high levels of "anxiety, stress, and panic" when triggered by the emotional weight of living with a chronic MS diagnosis; continued therapy is needed to help client continue to work towards her goal of lowering "anxiety, stress and panic" and increasing her knowledge and application of tools and resources.

Plan

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 12/11/2025 9:02 AM - 9:57 AM

Duration: 55 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

th. and clt. will continue exploring clt's. patterning interpersonally and intrapersonally, practice grounding and resourcing her "anxiety, overthinking, and self-consciousness" and work with her attachment patterning in relationships. Along with exploring clt's. psychological defense structures using PRI psych-ed, Parts work, and somatic exercises. th. will hold space for clt. to continue to explore and speak to clt's. experience with MS. Along with building clt's. capacity for increased internal resourcing to decrease clt's. reliance on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 12/11/2025 at 10:17 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 12/11/2025 at 6:08 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 1/8/2026 9:01 AM - 9:57 AM

Duration: 56 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Good

Judgment/Impulse Control: Good

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 1/8/2026 9:01 AM - 9:57 AM

Duration: 56 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Clt. reports "Wellbutrin 300 mg for anxiety PCP Christa Toomre is the prescribing physician." Clt. reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." Clt. reports "I also take a ton of Vitamins." Clt. reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

Clt. reports her anxiety is presenting as "frustration."

Clt. reports "I get so emotional and triggered by the dynamics."

Clt. reports "it feels like he is harboring past pain and resentment and I don't know how to shift out of that with him" clt. speaking to why her anxiety becomes heightened in the relationship dynamics.

Objective Content

Th. and clt. explored clt's. anxiety. Clt. shared relationship dynamics that heighten her anxiety. Clt. started to cry and expressed feeling "stuck" in the dynamics. Th. asked open ended questions, offered supportive reflections, and emotional validation. Th. and clt. explored how to decrease clt's. excessive worry and intrusive thoughts connected to the dynamics. Clt. stated feeling "frustrated and wanting to just move forward but not knowing how to." Th. provided psych-ed, reframes, grounding and resourcing and non-violent communication tools for clt. to practice and implement within the dynamics to decrease her "anxiety, frustration, and sadness." Clt. engaged with interventions used and expressed feeling "okay and better" after discussing the dynamics and how to regulate emotionally.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, IFS, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Practice Nonviolent Communication, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, Symptom Management, and Validation Exercises.

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease "anxiety, panic, and stress" connected to MS diagnosis. Client will decrease her distress level from a 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 1/8/2026 9:01 AM - 9:57 AM

Duration: 56 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

-
2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her NS in the moment. Client will be tracking her triggers to be able to resource and ground herself when triggered from a distress level of 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

3. Client will explore and use cognitive reframing around "obsessive/ ruminating thought" patterns that are currently at distress level of 10. Client will increase her awareness and ability to challenge her negative thought patterns and offer reframes to decrease distressing thought patterns from a level 10 to 9 over the next six months aeb client report in session.

Progress: Progressing

4. Therapist will provide client with PRI, attachment, parts work, and nervous system regulation psychoeducation and tools to use to increase client's awareness of the trigger's in her system and decrease client's "anxiety, overthinking, and self-consciousness" connected to interpersonal relationship dynamics. Client will decrease her distress level from a level 6 to a level 5 over the next 6 months aeb client report in session.

Progress: Progressing

5. Client reports "being told by doctors the physical manifestation of stress can have a negative impact on client's well being has led to an increase in "fear and worry" around MS diagnosis. Client will continue to learn and implement stress reduction tools and resources to decrease the "fear and worry" from a distress level of 10 to a 9 over the next six months aeb client report in session.

Progress: Progressing

6. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Clt. was at home for telehealth session.

Clt. still experiences high levels of "anxiety, stress, and panic" when triggered; continued therapy is needed to help client continue to work towards her goal of lowering "anxiety, stress and panic" and increasing her knowledge and application of tools and resources.

Plan

Th. and clt. will continue exploring clt's. patterning interpersonally and intrapersonally, practice grounding and resourcing her "anxiety, overthinking, and self-consciousness" and work with her attachment patterning in relationships. Along with exploring clt's. psychological defense structures using PRI psych-ed, IFS, and somatic exercises. Th. will hold space for clt. to continue to explore and speak to clt's. experience with MS. Along with building clt's. capacity for increased internal resourcing to decrease clt's. reliance on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 1/8/2026 9:01 AM - 9:57 AM

Duration: 56 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 1/10/2026 at 6:45 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 1/12/2026 at 6:48 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 1/15/2026 9:02 AM - 9:59 AM

Duration: 57 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Good

Judgment/Impulse Control: Good

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 1/15/2026 9:02 AM - 9:59 AM

Duration: 57 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Clt. reports "Wellbutrin 300 mg for anxiety PCP Christa Toomre is the prescribing physician." Clt. reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." Clt. reports "I also take a ton of Vitamins." Clt. reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

Clt. reports "in this dynamic neither of us are wanting to put an end to the relationship" clt. sharing another aspect of what keeps her stuck in the anxious pattern she spoke to.

Clt. reports "I have so much indecisiveness around this area of my life that I don't have in others and I think it's because I am not as emotionally invested in the other areas of my life." Clt. is able to see how she can be motivated and move from her emotions which is keeping her stuck in the anxious pattern of not choosing something different for herself.

Clt. reports "I am scared of losing the friendship and the comfort and it makes me sad."

Objective Content

Th. and clt. explored clt's. anxiety connected to relationship dynamics. Clt. shared feeling "confused and conflicted" regarding the dynamics which is leading to feelings of heightened anxiety. Th. asked open ended questions, offered supportive reflections, and emotional validation. Th. and clt. explored what patterns are leading to clt's. confusion and conflict. Clt. shared "fear, sadness, and jealousy" are emotions that increase her anxiety and motivate her to cycle in and out of the pattern of staying in a relationship that is longer serving her out of fear and loss. Clt. stated "I got back to what feels easier instead of staying with what feels harder." While clt. is aware of this pattern she can become over powered by her NS response and move from what feels easier in the moment. Th. is working with clt. on building her capacity for negative emotional discomfort. Th. provided psych-ed, reframes, grounding and resourcing tools, and validation and somatic exercises for clt. to practice and implement to decrease her "anxiety, fear, jealousy, and sadness" and shift out of this pattern that is no longer serving her. Clt. engaged with interventions used, took notes throughout session, and stated feeling "better and I am going to reflect more on what we talked about afterwards."

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, IFS, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Practice Nonviolent Communication, Preventative Services, Psycho-Education, Somatic Exercises, Structured Problem Solving, Supportive Reflection, Symptom Management, and Validation Exercises.

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease "anxiety, panic, and stress" connected to MS diagnosis. Client will decrease her distress level from a 8 to a level 7 over the

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 1/15/2026 9:02 AM - 9:59 AM

Duration: 57 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

next six months aeb client report in session.

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her NS in the moment. Client will be tracking her triggers to be able to resource and ground herself when triggered from a distress level of 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

3. Client will explore and use cognitive reframing around "obsessive/ ruminating thought" patterns that are currently at distress level of 10. Client will increase her awareness and ability to challenge her negative thought patterns and offer reframes to decrease distressing thought patterns from a level 10 to 9 over the next six months aeb client report in session.

Progress: Progressing

4. Therapist will provide client with PRI, attachment, parts work, and nervous system regulation psychoeducation and tools to use to increase client's awareness of the trigger's in her system and decrease client's "anxiety, overthinking, and self-consciousness" connected to interpersonal relationship dynamics. Client will decrease her distress level from a level 6 to a level 5 over the next 6 months aeb client report in session.

Progress: Progressing

5. Client reports "being told by doctors the physical manifestation of stress can have a negative impact on client's well being has led to an increase in "fear and worry" around MS diagnosis. Client will continue to learn and implement stress reduction tools and resources to decrease the "fear and worry" from a distress level of 10 to a 9 over the next six months aeb client report in session.

Progress: Progressing

6. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Clt. was at home for telehealth session.

Clt. still experiences high levels of "anxiety, stress, and panic" when triggered; continued therapy is needed to help client continue to work towards her goal of lowering "anxiety, stress and panic" and increasing her knowledge and application of tools and resources.

Plan

Th. and clt. will continue exploring clt's. patterning interpersonally and intrapersonally, practice grounding and resourcing her "anxiety, overthinking, and self-consciousness" and work with her attachment patterning in relationships. Along with exploring clt's. psychological

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 1/15/2026 9:02 AM - 9:59 AM

Duration: 57 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

defense structures using PRI psych-ed, IFS, and somatic exercises. Th. will hold space for clt. to continue to explore and speak to clt's. experience with MS. Along with building clt's. capacity for increased internal resourcing to decrease clt's. reliance on external resourcing and validation.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 1/15/2026 at 10:29 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 1/16/2026 at 6:47 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 1/27/2026 9:02 AM - 10:01 AM

Duration: 59 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Good

Judgment/Impulse Control: Good

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 1/27/2026 9:02 AM - 10:01 AM

Duration: 59 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Clt. reports "Wellbutrin 300 mg for anxiety PCP Christa Toomre is the prescribing physician." Clt. reports "MS medication is Ocrevus that's the infusion taken 2x a year administered in January and June/ July by Neurologist Dr. Dasari." Clt. reports "I also take a ton of Vitamins." Clt. reports "I am on Marina an IUD for birth control."

Subjective Report and Symptom Description

Clt. reports "I am feeling stronger and it is not as upsetting" clt. speaking to the decrease in her anxiety.

Clt. reports "people kept asking me if I was going to go and I wasn't feeling anxious but not I am and I am confused as what I should do."

Clt. expressed feeling conflicted and confused and asked th. for support in how to move forward. Th. worked with clt. on finding her own knowing around the choice.

Clt. reports "I have a fear of being put into a box" which is increasing her anxiety.

Objective Content

Clt. started by sharing she has another infusion on Thursday as a part of her MS continuous care. Clt. shared feeling "less anxious" this time around. Th. listened as clt. shared and celebrated clt. feeling more resourced regarding her infusion appointments. Th. and clt. then explored clt. decreasing her session frequency due to clt. feeling "more stabilized" and wanting to explore moving to every other week. Clt. was able to see her progress in stabilizing in her anxiety when she had to interface with an external stimulus that was once activated her into heightened states of anxiety; IE: her infusion appointments. Clt. then moved into sharing an area she is experiencing heightened anxiety around. Th. listened as clt. shared. Th. and clt. explored clt's. anxiety. Clt. shared feeling anxious of being misperceived and not knowing which choice to make. Th. offered observations, psych-ed, supportive reflections, emotional validation, reframes, and asked open ended questions. Clt. engaged with interventions used and expressed feeling "better" and having "clarity" regarding her decision.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, IFS, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease "anxiety, panic, and stress" connected to MS diagnosis. Client will decrease her distress level from a 8 to a level 7 over the next six months aeb client report in session.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 1/27/2026 9:02 AM - 10:01 AM

Duration: 59 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Progress: Progressing

2. Client will use self reflective activities such as journaling 2x a week to reflect and bring awareness to what is happening before, during, and/or after stress response is activated that leads to anxiety flare ups to help client understand what is happening in order to implement new coping skills and regulate her NS in the moment. Client will be tracking her triggers to be able to resource and ground herself when triggered from a distress level of 8 to a level 7 over the next six months aeb client report in session.

Progress: Progressing

3. Client will explore and use cognitive reframing around "obsessive/ ruminating thought" patterns that are currently at distress level of 10. Client will increase her awareness and ability to challenge her negative thought patterns and offer reframes to decrease distressing thought patterns from a level 10 to 9 over the next six months aeb client report in session.

Progress: Progressing

4. Therapist will provide client with PRI, attachment, parts work, and nervous system regulation psychoeducation and tools to use to increase client's awareness of the trigger's in her system and decrease client's "anxiety, overthinking, and self-consciousness" connected to interpersonal relationship dynamics. Client will decrease her distress level from a level 6 to a level 5 over the next 6 months aeb client report in session.

Progress: Progressing

5. Client reports "being told by doctors the physical manifestation of stress can have a negative impact on client's well being has led to an increase in "fear and worry" around MS diagnosis. Client will continue to learn and implement stress reduction tools and resources to decrease the "fear and worry" from a distress level of 10 to a 9 over the next six months aeb client report in session.

Progress: Progressing

6. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Clt. was at home for telehealth session.

Clt. still experiences high levels of "anxiety, stress, and panic" when triggered; continued therapy is needed to help client continue to work towards her goal of lowering "anxiety, stress and panic" and increasing her knowledge and application of tools and resources.

Plan

Th. and clt. will continue exploring clt's. patterning interpersonally and intrapersonally, practice grounding and resourcing her "anxiety, overthinking, and self-consciousness" and work with her attachment patterning in relationships. Along with exploring clt's. psychological defense structures using PRI psych-ed, IFS, and somatic exercises. Th. will hold space for clt. to continue to explore and speak to clt's. experience with MS. Along with building clt's. capacity for increased internal resourcing to decrease clt's. reliance on external resourcing and validation.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Amanda Duffy, DOB 1/14/1998

Date and Time: 1/27/2026 9:02 AM - 10:01 AM

Duration: 59 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Every 2 Weeks

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 2/1/2026 at 7:24 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 2/2/2026 at 8:30 PM.